NIHR | National Institute for Health and Care Research

NIHR Fellowships Programme

Funding, Support or Development Opportunity	2026/343 Predoctoral Award (Cohort 2)

SUMMARY INFORMATION

Contracting Organisation	OXLEAS NHS FOUNDATION TRUST
Partner Organisation(s)	Queen Mary University of London,
Start Date	01/09/2026
End Date	30/11/2027
Duration (months)	15
% FTE Commitment	40%
Percentage of award time undertaking practice/clinical work	0%
Total Research Costs Requested	£49,153.39
Has this application been previously submitted to this or any other funding body?	No
Partnership Fellowship	Not Applicable

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LEAD APPLICANT & RESEARCH TEAM

Lead Applicant

Full Name	Mrs Claire Higgins
Organisation	OXLEAS NHS FOUNDATION TRUST
Department	Children and Young People's Directorate
Professional Background	Allied Health Professional
Detailed Professional Background	Physiotherapist
Professional Registration	HCPC
ORCID	0000-0002-6229-0512
Research Degree Status	Registered
Contextual Factors	I am a physiotherapist and early career researcher with 27 years' clinical experience supporting non ambulant children with complex physical and learning needs. I work in local authority special schools and community NHS services, settings critically under represented in research. I focus on inclusion and equity, enabling physical activity participation through practical, replicable support. Alongside managing a complex caseload, I lead an integrated therapies team and provide clinical supervision, while caring for two teenagers in exam years, further constraining protected research time. Despite this, I strengthened my trajectory through an ICA internship (2023) and the NIHR Pre-Application Support Fund (PASF) award (2025–26), collaborating with my supervisors and lived experience advisors to build skills, networks and outputs. During my PASF I have addressed prior NIHR feedback on publication readiness and two papers are now near submission. I work in a research naïve community disability, alongside an underserved, non ambulant population with significant intellectual disability, often termed profound and multiple learning disabilities. I have embedded an assistive technology physical activity programme in the curriculum and built a strong methods base (Child Health MRes; doctoral skills platform). To progress to an inclusive doctorate, I need targeted training in co design with children, complex assent/dissent and ethics–risk governance, and implementation science, skills essential to deliver a feasible, ethically robust doctoral protocol and build capacity in these settings. A predoctoral award will enable this focused skills programme and the protected time necessary to complete the publications and preparatory work underpinning doctoral readiness. Without the resources provided by this award, there would be a significant risk in my ability to consolidate the scope of work already reached through the PASF and progress onwards to doctoral study.

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APPLICATION DETAILS

ODA Compliance
NA

Working with People and Communities Summary
I am structuring my strategy around Health Research Authority/INVOLVE guidance on how public involvement strengthens ethical quality (relevance, acceptability, informed assent, participation, communication). This is essential to support children with complex mobility/learning needs, whose experiences are shaped by intersectionality and geodemographic disadvantage. Acceptability must include children themselves, even when communication is complex. Through partnership working I will deepen understanding of ethical issues specific to this community. My preparatory work has two advisory groups: Children's Participation Lab (ChiP, led by Gimeno) and the Willow Dene school TRYcycle group. My physiotherapy experience grounds decisions in reality, and the pre doctoral award provides protected time for meaningful co production ahead of a doctoral fellowship.
Lived experience has already shaped direction. A parent co authored a conference poster describing cycling as 'the most important health activity' in her daughter's life; a young PPI advisor reframed language by describing physical activity as 'moving with intention'; and a third young person proposed a shift from 'performance to participation', raising the concept of 'best interest' decisions (for who?), and asking how children with limited energy are enabled to choose where to spend it. What we will explore (1): I will continue my partnership with the ChiP Lab, the advisory group for the NIHR Dystonia IMPACT and Co-SMART trial. Meeting every second month provides a structured route to generate ideas and widen perspectives across ages, mobility and communication profiles. I work one-day a week with a Young Advisor employed as a lived experience contributor. His social media and entrepreneurial skills expand team capability and support iterative feedback. We will use accessible materials and established NIHR reimbursement routes to support consistent involvement. One member has reviewed this submission, and a young person/parent will review the full doctoral application once drafted. (2) TRYcycle: In parallel, I will facilitate structured touchpoints in special schools to bring learning back into programme development. The focus is what matters to children during cycling and how this links to assent. Through delivering the programme daily, organising a face-to-face workshop, and facilitating peer advocacy cycle sessions, we will scope; what feels safe and enjoyable; how effort/comfort/pride are noticed; how pacing, positioning and peers influence participation; when a child wants to stop; and what outcomes look like beyond distance travelled/minutes cycled. Children and staff have already highlighted practical enablers such as quieter spaces, predictable routines, and trusted supporters. The advisory group will discuss multimodal routes of assent, explore signals of dissent, and consider school context feasibility. To evidence progress, I will keep a simple 'you said, we did' log, an experience check (clarity, comfort, choice), and meeting notes stating who was involved and what changes next.
My team strengthens this preparatory work. Dr Shepherd offers national expertise in research involving people with impaired decision making capacity. Professor Smith brings internationally recognised leadership in co production. Dr Gimeno the experience in research with children with complex needs. Their input will help ensure rigorous methods and clear documentation of influence across both strands.

SUPPORT AND MENTORSHIP

NIHR Fellowships Programme

Full Name	Ms Faith Brooks
Organisation	OXLEAS NHS FOUNDATION TRUST
Email Address	faith.brooks1@nhs.net
Role	Practice Supervisor
Description of Support Provided	My practice supervisor and line manager Faith Brooks is a Band 8b Speech and Language Therapist (HCPC/RCSLT) with 25 years' experience in paediatric early years communication and a dual clinical–operational leadership role at Oxleas. We meet weekly for a service lead huddle and formally for my supervision every six weeks. Supervision sessions are logged on the Trust learning platform, and we have an agreed ad hoc route for time critical issues. Faith's dual vantage point is central to my clinical academic trajectory. She mentors my day to day strategic and operational workstreams, supports workload balance and protected research time, and provides a clear escalation pathway through service leadership if pressures arise. Her clinical expertise directly matches my predoctoral/doctoral programme, which will explore how developmental profiles and communication needs shape participation in physical activity and daily living among children with complex mobility and communication needs. Through supervision, we will be able to discuss the progress of my tangible outputs and identify any facilitators or barriers as they arise. Faith will be able to connect me to Speech and Language Therapy networks internally and externally to help build the multidisciplinary framework, embedding communication goals and the life context definition of assent as the programme develops. As an operational lead, she will feed emerging findings into directorate priorities, enabling timely translation into quality improvement and service pathways, while ensuring Oxleas' host commitments are honoured through iterative feedback, check ins and formal progress reviews. This practice supervision complements academic supervision (methods and scholarly direction) by focusing on operational feasibility, workforce interfaces, and service adoption. Progress will be measured and reviewed quarterly ,giving NIHR a visible line of sight from supervision to service level impact.

Full Name	Dr Hortensia Gimeno
Organisation	Queen Mary University of London
Email Address	h.gimeno@qmul.ac.uk
Role	Primary Academic Supervisor
Description of Support Provided	I am a Children's Clinical Academic Occupational Therapist, Clinical Reader in Child Health at Queen Mary University of London (QMUL) and Associate Director for Therapies Research at Barts Health. I currently hold a NIHR Advanced fellowship, and I exemplify the type of clinician the NIHR seeks to nurture and amplify. My career demonstrates that world-class, theory-driven research can

be generated from within clinical practice, and that AHPs can lead interdisciplinary programmes delivering impact for patients, services, and the wider research ecosystem. Through establishing joint clinical academic posts, supporting progression from pre-doctoral to post-doctoral levels, and embedding AHPs within interdisciplinary research teams, I have contributed to a step-change in how clinical academic careers are realised in AHP research and have supported over 20 AHPs on pre-doctoral, doctoral, and post-doctoral fellowships. Since completing my PhD in 2019, I have developed an internationally competitive research portfolio despite highly constrained research-protected time. As such I am well versed on what the challenges are to develop a research career when there is little infrastructure around you or when working in naïve research environments.

I have a national and international profile in leading research for childhood-onset complex movement disorders. My primary research programme addresses critical gaps in paediatric neurorehabilitation, particularly for children with rare conditions, intellectual disability, and childhood-onset movement disorders who are frequently excluded from research. I have led the development and evaluation of theory-driven, person-centred rehabilitation interventions targeting activity and participation outcomes meaningful to children and families, generating some of the first high-quality intervention evidence in this field (Neurology; Developmental Medicine & Child Neurology). Since September 2024, I have built and lead a children's health research group comprising seven AHP researchers, methodologists, and co-researchers with lived experience, enabling inclusive research with underserved populations. This work has informed clinical pathways, contributed to international clinical practice guidelines, and expanded into policy through my role as co-applicant and work-package lead on an NIHR Policy Research Programme grant addressing workforce modelling for paediatric therapists supporting children and young people with special educational needs and disabilities (SEND).

My movement disorder expertise, including cerebral palsy, current research program, and trajectory as a clinical academic occupational therapist expertly positions me to support Claire's fellowship. I am deeply committed to increasing allied health professionals' capacity and capability for research, and Claire and my research interests to ensure children's voices are heard, particularly those currently under-represented in research, align and are complimentary.

Academic experience: I hold an NIHR Advanced Fellowship (2024-2028) to develop a research program for children and young people with movement disorders. My research, rooted in a solid theoretical and clinical foundation, aims to facilitate the optimal participation of children in daily life.
Throughout my research career, I have demonstrated a dedication to understanding and addressing real-life problems. My experience, spanning over 28 years in the NHS, reflects my commitment to embedding research in clinical practice and utilising routinely collected data to enhance the evidence base. This aligns with the origin, aims and long-term objectives of Claire's TRYcycle research plan.

	I have received over £7million in previous research funding through funders such as EU Horizon 2020, Rosetrees Trust, Barts Health Charity, and my previous NIHR Clinical Doctoral Fellowship. I am especially interested in individual trajectories and employ innovative research methodologies, such as single-case experimental design/n-of-1 trials, to measure intervention outcomes, particularly for populations underserved in research. Children with complex movement disorders are an underserved population in research and I am a passionate advocate of advancing research studies in this domain. This directly maps to Claire's focus as a clinical academic and affords her a strong clinical bridge between health and the school environment in which she is situated. I am well positioned to support Claire in developing her own fellowship application, particularly as an experienced AHP researcher. Claire and I have been working closely on developing her research skills and I have provided learning and research hands-on experience for the past few years. More recently I have supported Claire closely throughout her NIHR awarded Pre-Application Support Fund (PASF) and she has an honorary appointment at QMUL with my group. During the pre-doctoral fellowship I will provide academic supervision and valuable advice on developing Claire's clinical academic career, both during and post-fellowship. I will support in day-to-day aspects of the research, and I will meet Claire every other week for supervision, and monthly for joint sessions with Professor Brett, Claire's secondary supervisor. Research Environment: Claire's fellowship will be hosted at Oxleas NHS Trust with an honorary contract at QMUL Centre for Bone and Joint Health (the ChiP Lab). Claire will benefit from being embedded in this group with other clinical academic AHPs, such as Dr Lisa Newington (clinical academic senior lecturer hand therapist), Professor of Rehabilitation and first physiotherapist to be awarded a UKRI Future Leaders Fellowship Dr Katie Sheehan and other AHP PhD students. This group has strong links to the Pragmatic Clinical Trials Unit from QMUL. During this pre-doctoral fellowship, I will support Claire to develop collaborations she will have the opportunity to be involved in my program of research to enhance her skills and knowledge. Established Relationships: I have been working with Claire during her HEE internship, Pre-application support fund, and developing this pre-doctoral application. We have had regular discussions on the skills Claire needs to develop and refine her research questions and methods. I have already provided Claire with opportunities, such as nominating her to present at the National Posture and Mobility Group conference in 2022. I have met regularly with Claire, together with Professor Bret Smith (my secondary supervisor) to develop this pre-doctoral application.

Full Name	Professor Brett Smith
Organisation	DURHAM UNIVERSITY
Email Address	brett.smith@durham.ac.uk

NIHR Fellowships Programme

Role	Academic Supervisor
Description of Support Provided	Professor Brett Smith is Professor of Disability and Physical Activity at Durham University. His research expertise blends disability studies, public health and qualitative methodology, with a sustained record of co-produced programmes that move from evidence to policy and practice. Brett is an author of the first UK Chief Medical Officers' Physical Activity Guidelines for disabled children (2022), which was co-produced with disabled children, families and practitioners. His work on the government guidelines is foundational to my research. Academic Studentships and Experience: Brett has supervised over 50 MSc students and 19 PhD students to completion and externally examined 41 PhDs internationally. He is a leading authority in qualitative methodology, founding the International Society of Qualitative Research in Sport and Exercise and co-editing the Routledge Handbook of Qualitative Research in Sport and Exercise. Current Research and Alignment with My Work: Brett leads Moving Social Work, a multi-year, co-produced research and implementation programme with Disability Rights UK and Sport England (phase two funded at ~£1m) that designs, tests and scales resources embedding physical-activity advocacy into social work education and practice through staged co-design and national rollout. This programme has also produced peer-reviewed guidance on co-production in practice (typology, working principles, quality criteria), which I will translate directly into my consent-enablement work. National/International Leadership: Brett's guideline work exemplifies policy-level co-production, from evidence review to co-produced infographic and animation with disabled children and families, demonstrating how participatory design improves message relevance and sector adoption. He co-leads Durham's Narrative Practices Lab (Wellcome Discovery Research Platform), using narrative and lived experience to co-create health and social-care tools. Research Environment and Support: Durham University offers a strategic, research-intensive environment spanning 27 academic departments and over 40 research institutes, with Brett's affiliations including the Durham Research Methods Centre and the Wellcome-funded Narrative Practices Lab. This affords me access to transdisciplinary methods, policy partners and knowledge-mobilisation pathways across health, education and social care. Bespoke support Plan: As my secondary supervisor, Brett will meet with me monthly (virtual) and host an in-person placement with his team. Brett's supervision will ensure that the programme is built on co production as a normative stance, not a technique, sharing power, jointly owning decisions, and valuing lived and professional knowledge as equal throughout the full research cycle. He will help me develop the ground rules that operationalise these principles (joint decision rights, reciprocal benefit, continuous dialogue, relationship based working). I will be supported to develop assent principles as an ongoing process shaped by relationships and context, ensuring inclusive voice, fair recognition, and ethical knowledge generation. This stewardship ensures that co production underpins, guides, and shapes the foundations of all aspects of the programme going forward. This ranges from who decides what counts as assent, to how findings are owned, authored, and implemented.

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SUMMARY BUDGET

Justification of Costs

This Fellowship is the necessary bridge to a high quality doctorate. My previous training and MRes have strengthened my trajectory, but I still require focused development in: ethics and governance for research with children with complex needs; inclusive, co produced methods for assent/shared decision making; applied designs for complex studies; and guideline translation and implementation. Activities will map to the Vitae researcher framework to target competencies and evidence progress.

Type of cost: Salary back fill £46,088.39 40% FTE, 15mth programme. Costed at Band 8a Physiotherapist. YEAR 1: Salary £25,900.40; Higher cost of living support £2454.80; Contracted National Insurance £3953.20; Pension contribution £4077.60= £36,386. Divided in to cost per month= £3032.17 Sept 2026-March 2027 = 7 x £3032.17= £21,225.19 YEAR 2: (with 2.5% uplift added in line with NHS planning assumptions) Salary £26,547.60; Higher cost of living support £2516; Contracted National Insurance £3953.20; Pension Contribution £4,179.20 = £37,294.80. Divided in to cost per month = £3107.90 April 2026– November 2027= 8 x £3107.90= £24,863.20 TOTAL YEAR 1 £21,225.19 + YEAR 2 £24,863.20= £46.088.59.

The fellowship will provide the protected time to close remaining skills gaps and plan a competitive doctoral application. It will develop self management through practical experience balancing clinical and academic demands, and deliver tangible outputs: consolidation of first author publications and expand work with people and communities.

Type of cost: Bespoke masters level training £1820 Co-producing and co-designing health & social care interventions Level 7 | King's College London My context demands participatory methods that include non verbal children. This programme provides mentored, advanced skills in co design and practical use of the Medical Research Council/NIHR complex interventions framework. It strengthens the methodological competencies for my assent framework, closing a defined skills gap not covered in my prior study (3mths self-paced course on-line).

Type of cost: Supervisory £245: Research visit to placements at neighbouring Innovation and Implementation Research group (Northumbria University) and Professor Smith in the Department of Sport and Exercise Sciences (Durham University). Protected time to collaborate with experts and research teams will embed practical learning, mobilise the theoretical training gained through the award, maintain my established networks, and support a second presentation to the Innovation and Implementation Group. Travel costs: Train London to Newcastle single fare (£50); Train Newcastle to Durham single fare (£5); Train Durham to London (£50): Accommodation, 1 night stay between sites £140.

Type of cost: Conference (£500) Attend the European Academy of Childhood Disability (EACD) 2027 conference, held in Greece. This is the most relevant international forum for my field. The NIHR contribution will cover the registration fee; travel and accommodation will be funded through my employer's CPD budget, ensuring full resourcing of the trip. Attendance will enable me to present my work, develop clinical and research skills, and build international collaborations with childhood disability experts, supported by Dr Gimeno.Type of cost Working with People and Communities (£500): Co produced engagement with children/young people and partners to shape my doctoral proposal. The plan comprises two distinct activities:1) Workshop (Willow Dene School): Purpose: Co develop and refine the ethical approach, assent processes, and feasibility considerations for my doctoral study. Format: Face to face workshop (11:00–12:30) co facilitated by a young person from the ChiP Lab advisory group (QMUL) and myself. Participants: Three children/young people and their partners. Venue: Willow Dene School, no venue cost: Partner recognition payments £27.50/hour × 1.5 hours = £41.25 per person, 3 partners = £123.75: Co production hospitality: 8 people × £10 (3 CYP + 3 parents + 2 facilitators) = £80 (provided by the school enterprise 'Delish'): Family travel £40 per family × 3 = £120: Young person co facilitator (paid by ChiP Lab, no cost), accessible taxi £50: Workshop subtotal= £373.75

(2) Partner review of doctoral application: One parent and one young person to proof the doctoral application for clarity and scope. Budget: £55 × 2 = £110 Review subtotal: £110

Total spend: £483.75. Balance/contingency: £16.25 (variance in travel/hospitality or to top up recognition

payments)

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ACKNOWLEDGEMENT AND CONFLICTS

I have read and understood the terms of applying as Lead Applicant and accept this role	✓

Potential Conflicts	I have commercial sponsorship contribution towards the cost of my MRes studies at University College London from Motomed, manufacturers of Medimotion assistive technology.

Lead Applicant:	Claire Higgins	**Application Number:**	NIHR516523
Contracting Organisation:	OXLEAS NHS FOUNDATION TRUST		
Proposed Start Date:	01/09/2026	**End Date:**	30/11/2027

	Year 1	Year 2	Total Cost to NIHR
STAFF COSTS			
Lead and co-applicants	£21,225.19	£24,863.20	**£46,088.39**
Research staff	£0.00	£0.00	**£0.00**
Other staff	£0.00	£0.00	**£0.00**
Total staff costs	**£21,225.19**	**£24,863.20**	**£46,088.39**
OTHER RESEARCH/ AWARD COSTS			
Travel and subsistence	£245.00	£0.00	**£245.00**
Equipment	£0.00	£0.00	**£0.00**
Consumables	£0.00	£0.00	**£0.00**
Knowledge mobilisation, dissemination and impact	£0.00	£500.00	**£500.00**
Training and development	£1,820.00	£0.00	**£1,820.00**
Research inclusion	£0.00	£0.00	**£0.00**
Working with people and communities	£500.00	£0.00	**£500.00**
Other direct costs	£0.00	£0.00	**£0.00**
HEI indirect costs	£0.00	£0.00	**£0.00**
Other partner organisation indirect costs	£0.00	£0.00	**£0.00**
TOTAL COST TO NIHR	**£23,790.19**	**£25,363.20**	**£49,153.39**

Co-funding from other sources			**£0.00**
Cost to NIHR			**£49,153.39**
NHS support costs	£0.00	£0.00	**£0.00**
NHS excess treatment costs	£0.00	£0.00	**£0.00**
Non-NHS excess treatment costs	£0.00	£0.00	**£0.00**
TOTAL FUNDING			

Application Budget: NIHR516523

REQUESTED	£23,790.19	£25,363.20	**£49,153.39**

Narrative CV template for Personal Career Development Awards

****Module 1 – Contributions to the generation of new ideas, tools, methodologies, or knowledge**** (i) TRYcycle dataset (assisted cycling). I built and lead a special schools physical activity programme that has generated >4,300 cycle sessions for 74 non-ambulant children with complex motor and learning needs. This population is largely absent from the trials informing national physical activity guidance for disabled children and my dataset provides a rare opportunity to redesign intervention delivery. The retrospective cohort analysis of this routinely collected cycle data (10-year capture) is the source for my current MRes dissertation (complete August 2026).

(ii) First-author publication preparation (NIHR PASF enabled): (a) Content analysis of UK physical-activity guidelines, evidencing a representation gap for children with complex motor and learning needs (b) TRYcycle outcomes analysis (assisted cycling) demonstrating measurable physical capability in the same group. Target journal: *Developmental Medicine & Child Neurology*, a leading paediatric neurodisability journal with a global clinical readership and a strong record in publishing physical activity research. Submissions are planned for quarter one of the fellowship.

(iii) Knowledge resources Association of Paediatric Chartered Physiotherapists (APCP £10k bursary). I secured funding to deliver a clinician-facing Evidence Synthesis Guide web resource (link: Evidence Synthesis Guide | Association of Paediatric Chartered Physiotherapists) and an exemplar postural-care scoping review. Protocol accepted for the APCP Journal; abstract accepted for the European Academy of Childhood Disability (EACD) 2026 conference. These outputs strengthen research integrity by equipping physiotherapists to synthesise/appraise evidence and by modelling evidence-based practice.

(iv) Honorary Research Fellow, Queen Mary University of London — Dystonia IMPACT (NIHR Advanced Fellowship). I work alongside Dr Gimeno, my supervisor, and Chief Investigator, on an international childhood-onset movement disorders programme. I am site Principal Investigator for Oxleas NHS foundation trust, have co-facilitated a child interview, and contribute to the Young Persons Advisory Group. Working with Dr Gimeno on an international programme has strengthened my scoping-review, trial-operations and qualitative skills and informs my research. NIHR PASF time sustains my involvement (*Sept 2025–present).*

(vi) National methodological alignment (NIHR208347): As an external contributor, I support a national development grant to define a standardised, integrated health-education-social-care model of care for children with neurodisability. Invited as a contributor to represent non-ambulant children with complex learning needs, I have provided structured feedback on the national professional questionnaire and flagged inclusion/ measurement issues to ensure this population is represented (*ongoing*).

****Module 2 – Contributions to the development of others and maintenance of effective working relationships**:** (i) Leadership & supervision: As operational lead

for an integrated, multidisciplinary Special Schools team of 20 staff (Physiotherapy, Occupational Therapy, Speech & Language Therapy), I provide clinical and operational supervision. I have led the team through two successful tenders to sustain borough-wide services, strengthening relationships with schools, local authority and commissioners and creating stable conditions for staff development.

(ii) Data skills development: With Oxleas Business Informatics I am devising and implementing embedded capacity/demand and standardised workflow capture in our electronic records system. This is developing the wider integrated therapies team (75+) by equipping them to harness routine data for audit and service improvement.

(iii) Clinical development: I devise, coordinate, and teach the APCP Neurodisability Foundation Course, upskilling the next cohort of paediatric physiotherapists in evidence-based assessment, management, outcome measurement, and goal setting for complex disability. This sets a national baseline for competence and builds workforce capability across services.

(iv) Education development: I authored and implemented a risk-assessment and competency framework that has led workforce training, enabling teaching staff to deliver safe, supported cycle sessions during the school day. This has delivered sustainable programme scale-up across 2 schools; brought 11 bikes into routine use, trained 100+ staff, expanded physical activity opportunities for 140+ children; enabled 8,900 cycle sessions and integrated research-ready data pathways.

(v) Developing research capacity: I supervise an NIHR Health and Care professional intern within the *Dystonia IMPACT* research team, teaching independent screening and coding framework application within my content-analysis workflow. The process strengthens inter-rater rigour and reproducibility, demonstrating how my programme of work now trains others in method and practice.

(vi) Research culture building: Within my NHS trust, I act as a research champion. I am presenting in the upcoming “Research roadshow,” showcasing practical NIHR pathway steps, tools, and lessons learned, and signposting opportunities for underrepresented allied health professionals.

****Module 3 – Contributions to the wider research community**:** (i) Chair, Association of Paediatric Chartered Physiotherapy Neurodisability Committee (2024–present): I chair the APCP quarterly national neurodisability committee. In this role I led a Physical Activity symposium for non-ambulant children at national conference (2025). I establish thematic workstreams with topic advisers, linking clinicians to research active peers to build a community of practice. As chair, I also sit on APCP’s National Committee, embedding neurodisability priorities in the five-year plan. These calls translate into guidance and shared methods that enable practice and measurement standards, and a stronger evidence base for practitioners and the profession.

(ii) Professional guidance and knowledge mobilisation: I serve on the APCP working party updating the 2016 '*Working with Children'* guidance for paediatric physiotherapists, contributing to shared standards in practice and signposting professionals to the most relevant policies/guidelines. I am developing skills in guideline methods; cross-sector policy signposting; legal/ethical framework synthesis; evidence appraisal/consensus; and plain-language implementation *(work ongoing).*

****Module 4- Contributions to broader society**:** (i) System-level implementation**:** I prioritise service-user engagement (see *Working with People & Communities*) and align partners to deliver visible, auditable change across multi-agency programmes. I have implemented a borough-wide Key Working programme and Greenwich 'MOVE' (a mainstream school physical activity programme for disabled children) both sustained by local authority funding. This approach targets an underserved group, builds interdisciplinary partnerships, and embeds shared language and inclusive measures. In parallel, I support communities to identify and evidence outcomes that matter to disabled children and families, strengthening equity and participation.

(ii) Research–practice partnerships**:** I collaborate with physical activity charities (The Daily Mile) and industry partners (assistive technology) to translate research into real-world benefit. These partnerships strengthen cross sector links, enabling adoption and spread of evidence-based practice beyond academia, expanding opportunities for disabled children with complex needs.

****Module 5 – Suitability for this award**:** I have fully leveraged funding and training to date. I have run an MRes dissertation, an NIHR award, and my clinical/operational role alongside full-time parenting, demonstrating the capacity to deliver at pace. I now work across multi-site London pathways with cross-institutional supervision and hold a national platform for knowledge translation. My work bridges health, education, social care, and research. By embedding routine outcome data in physical activity practice, I have shown what works for non-ambulant children with complex learning needs in real settings and translated findings into training, frameworks, and research plans. I have documented their persistent under-representation and, critically, the specific barrier of assenting to participation with impaired capacity.

Adult impaired-capacity frameworks exist (e.g., NIHR INCLUDE), but there is no comparable framework for children with the most complex needs. In a future doctoral programme, I will develop and test assent methods co-designed with children, families, educators, and clinicians, aiming to improve inclusion, trial quality, and care for those with the greatest health burden yet least representation. A Predoctoral Award is time critical at this stage. It will secure publications, strengthen a competitive doctoral application, and move inclusion for non-ambulant children from policy to practice. It will build the methodological foundations for my PhD:(i) co-produced approaches to assent in complex neurodisability; (ii) UK governance for children who lack capacity, including assent-capture and ethics plans; (ii) inclusive outcome and measurement design; and (iii) process evaluation to define feasibility and acceptability metrics for future trials.

****Research Area of Interest****

An underserved population and a missing pathway: Non-ambulant children with complex motor and learning needs can be sedentary for up to 99% of their day (Verschuren 2016). Excessive sitting leads to early mortality (Levine, 2015) and has significant health implications for a population with multiple co-morbidities. Recognising movement as a mitigating factor and a fundamental human right, the challenge is to offer safe, measurable opportunities for physical activity.

Through my TRYcycle programme, I have shown that assistive technology can make participation accessible, with meaningful, curriculum embedded cycling sessions in special schools. However, wider uptake is constrained by a critical barrier: there is no practical, standard procedure to recognise and document a child's assent, dissent, or supported decision-making *during* cycle sessions, given their highly complex and diverse communication profiles. Securing each child's assent/dissent is critical because, ethically, children have a right to be heard in decisions that affect them and should be actively involved in a manner suited to their capacity. UK guidance expects researchers to seek assent and respect refusal, alongside parental consent where required. From an implementation perspective, acceptability is a core outcome for scale-up and by definition must capture the views of intervention recipients, not just adults around them. Without observable, documented assent, we cannot validly assess acceptability, and scale-up risks poor uptake, weak fidelity, and avoidable inequity. Building supported assent into routine delivery is therefore both an ethical safeguard and an implementation enabler. Unless ethical participation is established as a foundation, we risk creating another intervention that is done *to,* not *with*, children.

My work directly supports NHS England's priority to embed physical activity across care and reduce inactivity, especially for disabled children, and aligns with NIHR's INCLUDE expectation to design studies that include under-served groups. It responds to community priorities identified through the James Lind Alliance Childhood Neurological Conditions priority setting partnership (2022), which called for more research into non-medicinal interventions for children with motor disorders.

Increasing participation requires confronting underrepresentation. My content analysis of the UK Physical activity guidelines for disabled children shows that children with the most complex motor/learning needs are absent from the underpinning evidence. In a future doctoral programme of research, I will design and operationalise an assent enablement framework for school physical activity that captures observable assent, dissent and supported decision-making for non-ambulant children, so participation is ethical, measurable and scalable. This will protect rights, surface children's own preferences in routine/research data, and enable valid acceptability and scale-up, advancing public-health inclusion and safer, more efficient care.

Career Development Programme

Development programme rationale: Children and young people who are non-ambulant with complex learning needs remain largely invisible in the evidence underpinning physical activity guidance. My research shows that assistive technology can enable meaningful physical activity participation however, ethical scale-up beyond a solely "best-interests" model requires observable assent as the primary indicator of acceptability for each child. Through the Predoctoral Award, I will work together with children, families, and education staff to map "assent enablement" needs within a real-world intervention. I will register a scoping review protocol on approaches to identifying, supporting, documenting, and evidencing assent/dissent, and submit two manuscripts (1) A content analysis of the UK physical activity guidance for disabled children; (2) TRYcycle outcomes (quantitative data analysis). These outputs underpin my future doctoral framework. A bespoke training plan in co-production and assent, beyond MRes coverage, is essential.

****Training courses**** **(i)** **Co-producing and co-designing health & social care interventions Level 7 | King's College London** **(3-month self-paced course, 9 sessions of taught content x 2 a month):** My school-based, complex disability context demands participatory methods that include non-verbal children and avoid tokenism. This mentored Master's module delivers advanced co-design training to support safe, ethical scale-up of physical activity. It applies the Medical Research Council/NIHR complex-interventions framework (programme theory, context, early stakeholder engagement, and iterative development) as I prototype an assent framework for observable agreement. Advanced methods, multi-stakeholder facilitation, and practical problem solving outcomes align with my research, and the assessment, a participatory methods funding proposal, will directly build my doctoral plan. This targeted training closes a defined skills gap not met by my prior study.

(ii) Protected time will allow me to complete core, free training in ethical and inclusive research: the NHS Mental Capacity Act e-learning (updated guidance on capacity, best-interests decisions, and research involving people who lack capacity); the Health Research Authority module *Research involving participants lacking mental capacity*; the NIHR CPD-accredited *Informed Consent with Adults Lacking Capacity* course; and the NIHR *Leadership and Public Partnerships* module. Together, these targeted resources will strengthen my approach to consent, observable assent/dissent, and governance, ensuring inclusive study design. I will dedicate ~1 hour per week of award time to complete these.

****Placements**** **(i) Dystonia IMPACT trial- Continuation of honorary research fellow at Queen Mary's University:** Continuing as a pre-doctoral fellow in Gimeno's lab will embed me within the team developing the Delphi phase for an international core outcome set in childhood-onset movement disorders. I will gain hands-on experience translating qualitative interviews and scoping reviews into Delphi items, mapping outcomes, recruiting, and retaining diverse stakeholders, running multi-round Delphi processes, and providing feedback to support consensus. The practical methods, governance, and day-to-day coordination skills are directly transferable to

the design and oversight of my assent framework, strengthening its rigour and feasibility. The placement offers real life exposure to paediatric neurodisability research, ensuring my programme meets doctoral standards from the outset. I will be embedded one day per week throughout the award. The flexible format allows learning alongside my supervisor and peers while progressing my own programme.

(ii) Durham University (Professor Brett Smith): This focused skills-development placement, supervised by Professor Smith, will provide practical training in co-production design and facilitation, the creation and quality assurance of inclusive assent procedures, and the translation of research into implementation ready sector guidance. Professor Smith's national leadership in disability, physical activity, and government-commissioned guidance offers expertise beyond my host/partner institutions, strengthening my methodological core and ensuring outputs meet recognised national standards for rigour and applicability.

****Building Collaborations****(i) ASSENT project: Support from Professor Karen Bunning and University of East Anglia's ASSENT team enables me to develop my knowledge of tested, auditable assent methods in contexts of limited capacity and communication. Drawing on their four-year programme (including recommendations aligned with the Mental Capacity Act Code of Practice), I will learn from the national programme of work and start to map relevance for children with profound motor and learning needs. My collaboration strengthens ethical rigour and feasibility and positions me to progress toward paediatric adaptation of assent-ready tools previously limited to adult contexts.

(ii) INCLUDE Impaired Capacity to Consent Framework - Capacity consent research: Support from Dr Victoria Shepherd (Cardiff University) will enable me to work with the NIHR INCLUDE Impaired Capacity to Consent Framework through a paediatric lens. Learning about the framework's implementation toolkit will strengthen my protocols and ethics application, address exclusion risk, and align my methods with current guidance on designing for under-served groups. Dr Shepherd's expertise in consent methodology and alternative decision-making models will provide practical oversight as I translate these principles to children's assent and dissent in physical activity, enhancing the production of robust governance and implementation materials.

(iii) Innovation and Implementation Research group (Northumbria University) I have presented my TRYcycle research to the Innovation and Implementation Research group and will maintain this connection as I develop my doctoral proposal. Their focus on implementation science and knowledge mobilisation from early scoping through feasibility and scale-up aligns directly with my plans. This link, enabled by my current MRes supervisor Dr Phil Harniess, gives me an interdisciplinary route to plug in targeted implementation support, especially for embedding assent processes into routine delivery and planning real world evaluation, while I draft my doctoral proposal.

(iv) Dr Marilyn Bradbury: Developing an intervention to reduce sedentary behaviour in non-ambulant young people with long-term disabilities. - NIHR Funding and Awards

Marilyn is the chief investigator of the 'Move more' study. I will be drawing on her extensive knowledge of the existing literature on reducing sedentary behaviour and promoting physical activity in children with complex physical disability.

(v) Rachel Knight Lozano: leading the CHESTI study: Core outcome set and measures of cHest hEalth in children and young people with cerebral palSy in the communiTy settIng - NIHR Funding and Awards). This collaboration will enhance my practical learning of qualitative research in an aligned population and prepare me for predoctoral/doctoral work on known implementation barriers and facilitators.

****Conferences**:** I will attend two conferences to disseminate early findings, gain expert feedback, and build collaborations essential for my future PhD. At both meetings, I will submit abstracts based on analysis of my TRYcycle dataset, which provides rare evidence on physical activity capability and participation among children seldom represented in the literature. **(i) European Academy of Childhood-Onset Disability (EACD) 2027:** I will target an oral presentation at EACD to position my work within the leading European research forum in childhood-onset disability. The congress centres lived experience and a lifespan lens. Presenting here will allow me to receive methodological feedback from global experts and build collaborations that will strengthen the design and reach of my work. **(ii) Association of Paediatric Chartered Physiotherapists (APCP) Conference 2027:** Attend and submit for an oral platform presentation. This conference will serve as the UK launch of my findings, targeting paediatric physiotherapists who are under-represented at EACD but are key users of my future doctoral outputs. Presenting at APCP will secure national clinical engagement and support the early development of implementation pathways within UK services.

****Other**** **(i)** As my research topic and ideas evolve, the career development opportunities provided by the predoctoral will enable me to work with my supervisors and collaborators to craft a competitive doctoral application. This will be a pivotal step in advancing my career as a clinical academic. **(ii)** Monthly meetings will be held with my full supervisory team, enhancing my critical thinking, problem-solving skills, and resilience. These hybrid meetings (virtual and in-person) will introduce me to valuable networks and key partners with local and national influence in disability PA, such as Sport England, Cerebral Palsy Sport, Activity Alliance, and CanChild. **(iii)** As placement supervisor, Faith Brookes, the Oxleas Service Manager for Children's Integrated Therapies and a Speech and Language Therapist, will honour host-statement commitments, provide mentorship in communication enablement for complex needs, and connect me to Speech and Language Therapy networks beyond my team. **(iii)** The 15mth Predoctoral Award will provide protected time to deliver first-author, peer-reviewed publications, addressing a critical gap in my portfolio. Focused analysis and writing time is essential to produce high-quality manuscripts, without this support, outputs would be delayed and career progression constrained. This award enables timely dissemination and accelerates my research trajectory.

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02/03/2026

Dear NIHR Pre Application Support Fund Committee,

As Head of Department at Blizard Institute and the Lead for the Childhood Participation Lab in the Centre for Bone and Joint Health, it is our pleasure to provide Claire Higgins with this Queen Mary University of London (QMUL) partner letter of support for her NIHR Predoctoral Fellowship.

I (Gimeno) have supported Claire as a research mentor including during her NIHR ICA program since 2022. During this time Claire has demonstrated immense potential for clinical academic leadership, particularly for someone working in a research naïve environment with little access to research infrastructure and support. She has secured funding to complete a part-time MRes, public and patient involvement work, grant seed funding for development of national e-learning research resources for clinicians, and was successful with a Pre-application Support Fund (PASF) for 2 days a week for 12 months.

It is my absolute pleasure to provide this letter of support for Ms Claire Higgin's application for a pre-doctoral application. Working clinically whilst completing an MRes part-time and given Claire's caring responsibilities, has meant the two days per week for her PASF award have been focused on developing her research skills as recommended in her previous pre-doctoral application when rejected (2024). She has thrived this year whilst completing her PASF and has contributed to a large qualitative study interviewing children and families, has been a local PI for this NIHR funded study, and has contributed to an international NIHR funded core outcome set for childhood dystonia. Given the opportunity and protected time, Claire has fantastic potential to become a paediatric clinical academic physiotherapist working in community and local authority environments addressing an unmet need among a population currently underserved by the National Health Service.

I am well placed to support Claire whose research interest aligns to my own programme which has focused on development and evaluation of complex interventions for the same population with funding from NIHR, Rosetrees Trust, and Barts Charity. She has an excellent opportunity to progress a locally delivered initiative into a robust national level research evaluation. The pre-doctoral fellowship will enable Claire to develop the design of her study with PPI involvement to design a rigorous doctoral program and establish a strategy for building her national and international profile.

We have established ways of working inclusive of monthly meetings, email communications, ad hoc sharing of opportunities, as well as shared working at conferences. Claire and I have met regularly over the past couple of years, and weekly over the last twelve months to discuss her career ambitions, likely doctoral programme, and the training needs she requires to deliver a methodologically sound and impactful programme.

If successful, Claire will continue to have an Honorary Research Assistant Contract at the Blizard Institute, QMUL. She will be based within the Children Participation Lab at the Centre for Bone & Joint Health. The Centre has approximately 120 research active staff and PhD students. Indeed, Bone & Joint Health is an ideal HEI partner for Claire's predoctoral training and subsequent doctoral programme with access to experts in child health research, quantitative and qualitative methodologies, data storage infrastructure, the development of complex interventions, and implementation science.

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Claire will join an incredibly supportive team and environment. To evidence this, in 2022, QMUL retained the HR Excellence in Research Award following a 10-year review by Vitae, demonstrating the commitment to implement the principles of the Concordat to Support the Career Development of Researchers. More recently, Queen Mary University of London awarded our group at Bone & Joint Health an 'Enhancing Research and Innovations Culture' award to create evidence informed career pathways for allied health professionals, driving a culture change in workforce diversification and championing allied health professional clinical academic careers.

Claire and I will establish a shared supervisor agreement at the outset, arrange regular meetings with wider supervisory support, and ensure active engagement with the fantastic network of externally funded predoctoral (n = 8), doctoral (n = 4) and postdoctoral (n = 4) clinical academic allied health professional researchers at Bone & Joint Health. She will have access to our integrated academic training programme for clinical academics new to research, our weekly team meetings, whiteboard sessions where funding proposals are critically reviewed by a multidisciplinary team, and our trial's theme meetings where trial design and delivery are troubleshooted with relevant experts.

I will support Claire with her pre-doctoral training plan, specifically, extending my expertise in PPI, research methods, complex intervention design and implementation, and child health research. I will support Claire with extending her research network, collaborating on research opportunities, and extending her profile in child health.

I will offer Claire bi-weekly supervision and help her develop through challenging her ideas, providing feedback on her application, and interview preparation. I have offered support to develop this application and will offer support throughout the pre-doctoral application duration for Claire to develop the skills required to apply for doctoral funding. Claire will attend group meetings scheduled weekly where the team discuss progress with the ongoing funded NIHR studies. We will work together closely to optimise opportunities that will position Claire as a competitive applicant for pre-doctoral funding at the end of this award.

We wholeheartedly encourage you to support Claire's application as a promising future clinical academic physiotherapist in childhood complex movement disorders addressing an unmet need among a population currently underserved by the National Health Service.

Your sincerely,

Professor Mary Collins
Professor of Virology
Director, Blizard Institute

Hortensia Gimeno
Clinical Reader in Child Health Research
NIHR Advanced Fellow
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26th February 2026

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To: NIHR Predoctoral Fellowships Committee
Re: Letter of Support: Ms Claire Higgins (Physiotherapist) – NIHR Predoctoral Fellowship Application

Dear Committee Members,

As Associate Director for Specialist Children's Services at Oxleas NHS Foundation Trust, I am pleased to offer my support for Ms Claire Higgins and her application to the NIHR Predoctoral Fellowship. Claire is an experienced clinician working with children and young people who have the most complex motor and learning needs. In that challenging context she has consistently shown her commitment to the clinical academic trajectory through curiosity grounded practice, disciplined methods development, and the resolve to persevere when time and resources are limited.

Oxleas and our research environment is well placed to host optimal support. Oxleas delivers mental health and community services across south-east London, and we are investing in a research culture that improves patient care and builds clinical academic careers. Over recent years we have strengthened our research team, refined governance processes, and broadened collaborations. We have successfully hosted NIHR fellows and recognise the value of targeted protected time, robust supervision, and practical support in translating research into service improvement.

Oxleas is dedicated to creating and maintaining an inclusive and supportive research culture for all. We value and support equality, diversity, and inclusion, ensuring that our research environment is equitable and accessible. We want our clinicians and staff

to feel empowered and supported to use innovation and research to drive improved outcomes for our patients. We want Oxleas to be known as a Trust that delivers research-informed care and have prioritised this in our Strategy 2024-2027. We are dedicated to maintaining the highest standards in all our research activities and have committed to a three-year continuous improvement research and innovation strategic workplan setting out clear objectives, improvement plans and measurable outputs aimed at expanding research support infrastructure and developing core research skills in our workforce.

We see the responsibility for training the next generation of research leaders as a joint enterprise with contracting organisations. Our support for Claire's Predoctoral award application is a testament to our long-term commitment to her research career development. We are confident that Claire's involvement in this award will not only advance her clinical specialism in supporting CYP with complex movement disorders but also contribute to the broader goals of improving health outcomes and fostering innovation in research.

Oxleas has a strong track record of supporting early career researchers through various initiatives, including bridging support and mentorship programs. We are committed to ensuring that Claire has access to these resources, which will be crucial for her transition to a predoctoral bridge and future doctoral training. Our integration with national and international research networks will provide Claire with access to a broader community of researchers and resources, enhancing the impact and reach of her work through engagement with our research active staff. Support is available through South London Regional Research Delivery Network, South London Partnership, South London Commercial Research Delivery Network, our network of academic contacts from a range of clinical specialisms and HEIs, working collaboratively with other trusts to increase visibility and reach, improve knowledge mobilisation and wider dissemination of her work and achieve her research objectives.

Claire's work sits squarely within our strategic aims for inclusive, research-informed care and she benefits from being embedded in services where change can be designed, tested, and implemented at pace.

Claire has incrementally developed her early research career under Oxleas support over the last 23 years. Faced with barriers experienced by non-ambulant children, especially those with profound motor and learning needs, she has not accepted that absence from evidence is inevitable. Instead, she has built a credible platform for change, turning questions from frontline practice into structured inquiry, strengthened her methods through formal training, and cultivated partnerships that bring breadth to her perspective. She has advanced publications toward submission despite carrying a complex caseload and senior operational responsibilities. She engages young people and families so that lived experience shapes methods and materials and she contributes nationally to professional guidance and collaborative programmes. Taken

together, these are the signs of doctoral readiness, and we have confidence in her potential to deliver.

Claire's proposed predoctoral focus, consent enablement for inclusive physical activity, addresses a pivotal and under-served challenge: how to recognise and document assent, dissent, and supported decision-making for children with complex communication needs during routine activity and in research. This work is directly relevant to safeguarding, ethics, and feasibility in school and community settings. It promises practical tools and language that clinicians, teachers and families can apply, while creating the conditions for more inclusive trials and service evaluations. For Oxleas, this is not an abstract benefit as it speaks to our everyday practice and to our responsibility to ensure that outcomes reflect what matters to children and families who are too often missing from measurement.

To enable the full benefit of the NIHR Predoctoral Fellowship, Oxleas will provide Claire with the following support:

1) Protected research time and backfill: We will release Claire from clinical duties in line with the fellowship terms and fully backfill her role to protect programme delivery and service continuity. Our plan will cover all arrangements to ensure her research time remains intact throughout the award.

2) Space, resources and digital access: We will provide appropriate workspace, IT equipment, and secure access to systems and data where required and approved. This includes informatics support for service-evaluation components that underpin preparatory work and help translate findings into practice.

3) Research governance and ethics facilitation: Our R&D Office will guide Claire through HRA approvals, local capacity & capability, data protection, and school-based governance requirements. We will help operationalise child-appropriate consent/assent/dissent processes that align with safeguarding and best practice across health, education and families.

4) Methods, supervision and analytic support: We will ensure continued access to administrative support and statistical advice via the Research & Knowledge Team and facilitate external supervisory links as needed to strengthen design, analysis, and reporting.

5) Leadership development and career progression: Claire will access Trust leadership opportunities (including project and change-management learning) and mentorship consistent with our clinical-academic pathway, supporting progression from predoctoral to doctoral training.

6) Networks, collaboration and dissemination: We will support Claire's engagement with regional and national research networks and with partner schools and community services, enabling knowledge exchange, dissemination and adoption of tools that emerge from the fellowship.

7) Equality, diversity and inclusion: We will foster inclusive co-design and participation, including routes for young people and families to contribute meaningfully to planning, materials, and evaluation. This reflects our Trust-wide commitment to equity in research and care.

We consider this fellowship an investment in both quality and inclusion within children's services. Claire's work already informs how we think about ethical participation and measurement for children with the most complex needs and NIHR support will accelerate that impact. We will help embed outputs, such as accessible information, and inclusive measurement approaches, into pathways and training for staff. Our previous support in funding Claire's MRes at UCL (2024–26) demonstrates our long-term commitment to her development and hosting this predoctoral fellowship is the logical next step.

In my judgment, Claire exemplifies the qualities NIHR seeks in predoctoral fellows. She draws insight from real-world practice, demonstrates intellectual independence, ethical sensitivity, and the sustained effort required to deliver results. She has earned the opportunity this fellowship provides. Oxleas NHS Foundation Trust will be proud to host her and will work with her to ensure the benefits are felt by the children, families and communities we serve.

Yours sincerely,

Logan Watkins Associate Director – Specialist Children's Services

Oxleas NHS Foundation Trust

Title: Predoctoral training and development plan – Claire Higgins Application reference NIHR516523 September 2026-November 2027

TRAINING AND DEVELOPMENT ACTIVITY: MONTH	Sep	Oct	Nov	Dec	Jan	Feb	Mar	Apr	May	Jun	Jul	Aug	Sep	Oct	Nov
On-line learning activities:															
NIHR Informed Consent with children															
NHS Mental Capacity Act e-learning programme															
HRA module on Research involving participants lacking mental capacity															
NIHR CPD accredited Informed consent with adults lacking capacity															
Disability Equality Scotland: Inclusive Communication Workshop															
Schedule 1hr per wk for any other related free online learning modules/content															
In person training course:															
Co-producing and co-designing health & social care interventions Level 7 (KCL)															
Supervision:															
Joint supervisory team meetings primary & secondary: one hour each virtual															
In person supervisory meeting with primary supervisor (weekly)															
Meet with Collaborators (Dr Bradbury, Rachel Knight Lozano, Prof Karen Bunning, Dr Victoria Sheperd)															
Update Industry & charity collaborators (Medimotion, The Daily Mile)															
Oxleas NHS Trust clinical supervision (practice supervisor) monthly															
Working with people and communities:															
Young Persons Advisory groups: QMUL Kids Lab															
Review of Doctoral award submission															
Face to face workshop (Willow Dene school)															
Research placement opportunities:															
Honorary research fellow: Dystonia IMPACT trial (1-day wk QMUL)															
Northumbria University: Implementation and Innovation research group															
Durham University: Moving social work visit (Professor Brett Smith) in person															
Conferences:															
European Academy of Childhood disability conference attendance															
Association of Paediatric Physiotherapy conference attendance															
Predoctoral bridge application:															
Compile doctoral award application															
Research outputs:															
Publication write up and submission															
Scoping review protocol															